Extending Fisher–KPP glioma growth with oxygen and necrosis: a reproducible BrainWeb simulation study and a framework for parameter inference

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Abstract

Background. Fisher–Kolmogorov–Petrovskii–Piskunov (Fisher–KPP) models describe glioma spread and growth but do not explicitly represent oxygen limitation or necrotic material. We developed a minimal coupled extension and an interactive workflow to examine these assumptions.

Methods. Tissue-dependent cell diffusion and logistic growth were compared with oxygen-limited growth and a three-field model coupling viable tumor, quasi-steady relative oxygen and immobile necrotic cell equivalents. Simulations used the BrainWeb normal phantom, an identical dense Gaussian seed and assigned biological parameters over 90 days. Conservative finite-volume transport, operator splitting and an elliptic oxygen solver were assessed using analytical limits, conservation checks, ablations and refinement. Measurement-based inference and independent validation were formulated as future protocols.

Results. On the 2 mm grid, the coupled model contained 2.426 billion viable cells at day 90, compared with 2.847 billion for Fisher–KPP, a 14.80% reduction. Volumes above 0.2 of the density scale differed by 1.85%. Minimum relative oxygen was 0.0804, and necrotic material totaled 2.930 million cell equivalents. Only three voxels exceeded the necrotic threshold of 0.2. Suppressing hypoxic death had a small effect on total viable burden, whereas removing burden-dependent supply suppression largely restored the reference burden. Time refinement reduced field errors, while the necrotic peak and minimum oxygen remained sensitive to spatial resolution.

Conclusions. Oxygen feedback can change the simulated interior substantially while producing similar threshold-defined extent. This is a reproducible computational comparison, not patient-specific parameter estimation or biological validation. Translation requires calibrated observation operators, identifiable parameter subsets, independent spatial measurements and withheld-data forecasts.

Keywords: glioma; reaction–diffusion; oxygen; necrosis; BrainWeb; parameter identifiability; MRI; validation.

1 Introduction

Reaction–diffusion models describe an expanding population through spatial dispersal and local growth. The Fisher–KPP equation emerged from Fisher and from Kolmogorov, Petrovskii and Piskunov [1, 2]. Related proliferation–invasion formulations describe glioma-cell migration through anatomically heterogeneous brain tissue. Distinguishing white- and gray-matter motility permits predictions of preferential spread even when local transport is isotropic [3].

A single viable-cell field omits microenvironmental processes that can distinguish an infiltrating margin from a poorly oxygenated interior. The histological illustration motivating this work, from Comba et al., emphasizes invasion routes, vascular niches, reactive cells and necrosis [4];

the present model represents only a small subset of that conceptual picture. Earlier mechanistic modeling connected oxygen, cellular phenotypes and necrosis to pseudopalisading patterns [5]. Our reduction does not reproduce that model or explicitly resolve pseudopalisades. Oxygen-dependent proliferation and transfer into a necrotic compartment introduce additional assumptions and parameters. A more elaborate model can reproduce more patterns without providing uniquely identifiable biological explanations. Model complexity, observability and prediction therefore need to be assessed together.

Recent work illustrates the opportunities and constraints. Hiremath et al. investigated identifiability and selection among glioma treatment models using noisy virtual cohorts [6]. Miniere et al. incorporated serial MRI into a two-species model for 21 patients receiving chemoradiation, comparing sequential updates with prediction from limited imaging visits [7]. Zhang et al. explored physics-informed inference from a single structural MRI snapshot using characteristic nondimensional parameters [8]. These approaches motivate explicit consideration of what observations constrain; they do not establish that every mechanistic rate is identifiable from one scan.

Measurement validity is an additional issue. Independent histological evaluation of MRI-derived cellularity maps in 2025 found different performance in treatment-naive and recurrent gliomas [9]. A 2026 test–retest study combined diffusion, perfusion and oxygen-enhanced MRI to construct repeatable habitats in a murine glioma model [10]. Repeatability supports longitudinal inference, while biological specificity requires separate evidence. Recent 2026 preprints investigate poromechanical forecasting, sequential biomechanical model selection, and biopsy-derived kinetic information [11–13]. Their preprint status and experimental scope are retained here.

The present work develops a compact hierarchy from Fisher–KPP to oxygen-limited growth and coupled viable-tumor–oxygen–necrosis dynamics. Its contribution is a reproducible comparison on a fixed anatomical phantom, interactive exploration, numerical assessment and a proposed estimation and validation framework. We ask how oxygen and necrosis alter the simulated interior relative to threshold-defined extent, which assumptions dominate that change, and what measurements would be needed to interpret the parameters biologically. The study does not claim a new biological law or clinical forecasting performance.

2 Methods

2.1 Study design and anatomical data

This deterministic study used the original BrainWeb normal phantom, with its noise-free, intensity-uniform T1-weighted image and discrete tissue labels [14, 15]. The input is an anatomical substrate, not a longitudinal glioma dataset. No tumor masks, oxygen measurements or patient outcomes were available for calibration. The native 1 mm volumes contain $181 \times 217 \times 181$ voxels. The primary simulation used a 2 mm grid of $91 \times 109 \times 91$ voxels; intensities were resampled linearly and labels by nearest neighbor. Arrays use canonical RAS coordinates. Gray matter, white matter and the phantom's glial class form the transport domain Ω. CSF and other tissues are impermeable in every matched comparison. Data preparation and checksums are in Appendix A.

2.2 Reference model: Fisher–KPP

Let $c(\boldsymbol{x}, t)$ denote viable tumor-cell density in cells/mm^3. The reference, M0, is

$$\partial_t c = \nabla \cdot (D(\boldsymbol{x}) \nabla c) + \rho(\boldsymbol{x}) c \left(1 - \frac{c}{K}\right), \qquad \boldsymbol{x} \in \Omega. \tag{1}$$

D is effective cell migration in mm^2/day, ρ is growth in day^{-1}, and K is a density scale. D is scalar with separate white- and gray-matter values; fiber-aligned transport is absent. Cell and, when present, oxygen transport have homogeneous no-flux boundaries. The matched reference uses the same seed, coefficients, anatomy and output times as the extension.

2.3 Oxygen-limited and three-field extensions

The coupled model, M2, adds relative oxygen availability $o(\boldsymbol{x}, t)$ and immobile necrotic material $z(\boldsymbol{x}, t)$ in tumor-cell equivalents/mm^3. Define

$$\phi = \frac{c+z}{K}, \quad g(o) = \min\left(1, \frac{o}{\theta_p}\right), \quad h(o) = \left[1 - \frac{o}{\theta_d}\right]_+, \quad p(\phi) = \frac{1}{1+\alpha\phi}, \tag{2}$$

where $[a]_+ = \max(a, 0)$ and $0 < \theta_d < \theta_p < 1$. The system is

$$\partial_t c = \nabla\cdot(D\nabla c) + \rho g(o) c\,[1-\phi]_+ - \delta h(o) c, \tag{3}$$

$$\partial_t z = \delta h(o) c - \lambda_z z, \tag{4}$$

$$0 = \ell_o^2 \nabla^2 o + p(\phi)(1-o) - \beta(c/K) o. \tag{5}$$

Oxygen is solved at initialization and after every cell step. Its quasi-steady treatment imposes faster oxygen equilibration than tumor evolution, an assumption not tested by these data. Normal-tissue metabolism is implicit in the baseline $o = 1$. Distributed restoration replaces explicit vessels, and p is a phenomenological supply coefficient rather than measured blood flow. Viable cells consume oxygen. Necrotic material affects supply and available growth capacity but consumes no oxygen.

The two-field model M1 sets $\delta = 0$ and starts with $z = 0$, so oxygen only modulates growth. M2 includes death transfer and optional clearance. All examples start with $z = 0$. With $\beta = 0$, oxygen is one and M2 reduces to M0 for uniform K and the stated initial condition. Setting $\alpha = 0$ holds restoration fixed while retaining uptake. These nested cases provide implementation checks and ablations.

K regulates births rather than imposing hard packing on $c + z$. Diffusion can enter an already occupied location. Consequently, z/K and ϕ can exceed one: z/K is not a physical necrotic volume fraction. No healthy-cell population, edema, angiogenic state, tissue displacement, directed hypoxic migration or treatment response is included.

2.4 Initialization, parameters and numerical experiments

The initial density was a tissue-restricted Gaussian centered at RAS $(-30, 25, 30)$ mm, with standard deviations of 6 mm in each direction, a peak of 80,000 cells/mm^3, and truncation at three standard deviations. This dense seed was selected to produce an oxygen-limited interior early in the demonstration. Simulations cover 90 days, saving every 3 days. All biological values in Table 1 were assigned working values, not fitted estimates or established patient-level priors.

The primary M0–M2 comparison used $\Delta x = 2$ mm and maximum $\Delta t = 0.25$ day. Additional 2 mm runs evaluated M1, M2 with $\alpha = 0$, and M2 with $\beta = 0$. Temporal assessment repeated M2 with $\Delta t = 0.125$ and 0.0625 day. Native-grid assessment repeated M2 and matched M0 at 1 mm with a 0.125-day step. These nine model/step/grid cases are controlled simulations, not biological replicates. The grid comparison changes discretized anatomy as well as the numerical mesh, and is not a pure spatial-order experiment.

Cell transport uses conservative face fluxes with harmonic interface coefficients. Explicit diffusion is followed by exact frozen-coefficient logistic birth, hypoxic-death transfer and clearance substeps. Oxygen uses a compact no-flux stencil and preconditioned conjugate gradients. The

Table 1: Assigned parameters for the primary comparison. All are illustrative. Numerical tolerances and initialization details are in the appendices.

Symbol	Interpretation	Value	Unit
D_{WM}	White-matter migration	0.5	$\mathrm{mm}^2/\mathrm{day}$
D_{GM}	Gray/glial-matter migration	0.1	$\mathrm{mm}^2/\mathrm{day}$
ρ	Growth coefficient	0.04	day^{-1}
K	Density scale	100,000	$\mathrm{cells/mm}^3$
ℓ_o	Effective oxygen exchange length	0.2	mm
β	Relative tumor oxygen uptake	4	dimensionless
α	Supply suppression by total burden	4	dimensionless
θ_p	Oxygen for unrestricted proliferation	0.3	relative oxygen
θ_d	Hypoxic-death threshold	0.1	relative oxygen
δ	Maximum death-transfer rate	0.5	day^{-1}
λ_z	Necrotic-material clearance	0	day^{-1}

method is first order in time. A diffusion positivity bound controls substeps but does not guarantee reaction accuracy. Details are in Appendix B.

2.5 Outcome measures and interactive visualization

We recorded viable count $N_c = \int_\Omega c\,\mathrm{d}V$, necrotic equivalents $N_z = \int_\Omega z\,\mathrm{d}V$, peak density, minimum tissue oxygen, and volumes above $c/K = 0.2$ and 0.8. Necrotic threshold volume uses $z/K \geq 0.2$; low-oxygen volume uses $o < \theta_d$. These are model-defined quantities, not validated MRI detection thresholds. With closed boundaries,

$$\frac{\mathrm{d}}{\mathrm{d}t}(N_c + N_z) = \int_\Omega \rho g(o) c\,[1-\phi]_+\,\mathrm{d}V - \lambda_z N_z. \tag{6}$$

Death transfer cancels. With zero clearance and matched initial states, the M0 viable-count minus M2 live-plus-dead-count difference measures the cumulative birth deficit within the numerical balance.

The Jupyter interface exposes biological and numerical controls and retains Fisher–KPP. Four fields share time and slice coordinates, with axial, coronal and sagittal views. Live 3D histories support arbitrary slicing; compact saved results retain seed-plane histories and final volumes. MP4/GIF exports share a frame schedule. A slower movie spans 20 seconds with a 6-second day-90 hold. Display settings do not alter fields. The paper uses lossless arrays and displays the full necrotic range.

2.6 Proposed estimation and validation protocol

No inverse problem was solved. For future observations, modality m at time t_j would be represented by

$$y_{mj} = \mathcal{H}_m[c_{\boldsymbol{\theta}}(t_j), o_{\boldsymbol{\theta}}(t_j), z_{\boldsymbol{\theta}}(t_j); \boldsymbol{\eta}_m] + \varepsilon_{mj}, \tag{7}$$

where $\mathcal{H}_m$ includes contrast, resolution, partial volume and the biological-state-to-measurement link. $\boldsymbol{\theta}$ includes biology and initialization; $\boldsymbol{\eta}_m$ contains observation parameters. Calibration must incorporate noise, registration and initial-state uncertainty. An objective and identifiability restrictions are in Appendix C.

The proposed sequence estimates a small growth–migration subset from serial MRI, introduces oxygen parameters when additional modalities support them, and calibrates cell-scale and necrosis mappings using independent samples. Numerical verification, synthetic recovery, withheld-time prediction, tissue agreement and external-cohort performance are evaluated separately. M0 and M2 must be fitted separately to identical calibration data before predictive comparison.

Table 2: Day-90 outcomes for 2 mm model ablations with a maximum 0.25-day step. $V_{0.2}$ and $V_{0.8}$ are viable-density threshold volumes in mL. Oxygen and necrotic state variables are absent from M0, indicated by dashes.

Model	$N_c/10^9$	$V_{0.2}$	$V_{0.8}$	$\min o$	$N_z/10^6$
M0: Fisher–KPP	2.847	49.752	1.944	—	—
M1: oxygen-limited growth	2.429	48.856	0.000	0.0962	0.000
M2: oxygen and necrosis	2.426	48.832	0.000	0.0804	2.930
M2, fixed supply ($\alpha = 0$)	2.829	49.744	0.976	0.2245	0.000
M2, zero uptake ($\beta = 0$)	2.847	49.752	1.944	1.0000	0.000

3 Results

3.1 Matched 90-day comparison

The coupled model produced a lower-density interior than Fisher–KPP, despite a similar spatial extent at the lower density threshold (Figure 1). At day 90, M2 contained 2.426 billion viable cells versus 2.847 billion in M0, a 14.80% reduction. The respective volumes with $c/K \geq 0.2$ were 48.832 and 49.752 mL, differing by only 1.85%. In contrast, the volume with $c/K \geq 0.8$ was zero in M2 and 1.944 mL in M0. Peak viable densities were 0.637 and 0.896 times K, respectively.

Minimum tissue oxygen in M2 was 0.0804. Necrotic material totaled 2.930 million cell equivalents, with $z/K \geq 0.2$ occupying 0.024 mL, or three 2 mm voxels. Low-oxygen volume ($o < 0.1$) was 0.104 mL. The dense initial seed was already oxygen limited; this experiment therefore does not demonstrate the spontaneous emergence of a macroscopic necrotic core from a dilute initial tumor. The tiny thresholded focus is not a resolved anatomical cavity. Moreover, the maximum $z/K = 1.226$ exceeds one, as permitted by the cell-equivalent formulation.

The viable-count gap between M0 and M2 was 421.24 million cells. Only 2.930 million cell equivalents were stored as necrotic material. With no clearance, the remaining 418.31 million difference in live-plus-dead burden is attributable to reduced cumulative births in the discrete balance. Thus, in this case, the smaller viable burden mainly reflects inhibited proliferation rather than death transfer.

3.2 Controlled model ablations

Table 2 and Figure 2 separate growth inhibition, death transfer and the chosen supply feedback. Removing hypoxic death (M1) increased day-90 viable count by approximately 0.14% relative to M2. In contrast, removing burden-dependent suppression of oxygen supply ($\alpha = 0$) gave 2.829 billion viable cells, approximately 0.62% below M0. Its minimum oxygen, 0.2245, remained above the death threshold, and no necrotic material formed. The supply closure therefore has a much larger effect on this particular comparison than the death term. This finding is conditional on the assigned parameters and dense seed, and does not establish the closure biologically.

Setting uptake to zero ($\beta = 0$) maintained $o = 1$, yielded no necrotic material and reproduced the M0 solution. The largest final density discrepancy divided by K was below 10^{-5}, consistent with finite-precision differences between the numerical paths. This is a limiting-case verification, not an independent biological comparison.

3.3 Numerical verification and resolution dependence

The software test suite passed all 35 tests. Checks include analytical logistic behavior, diffusion conservation, boundary behavior, positivity, death-transfer balance, oxygen bounds and model limits (Appendix B). These results verify implemented properties without validating the biological

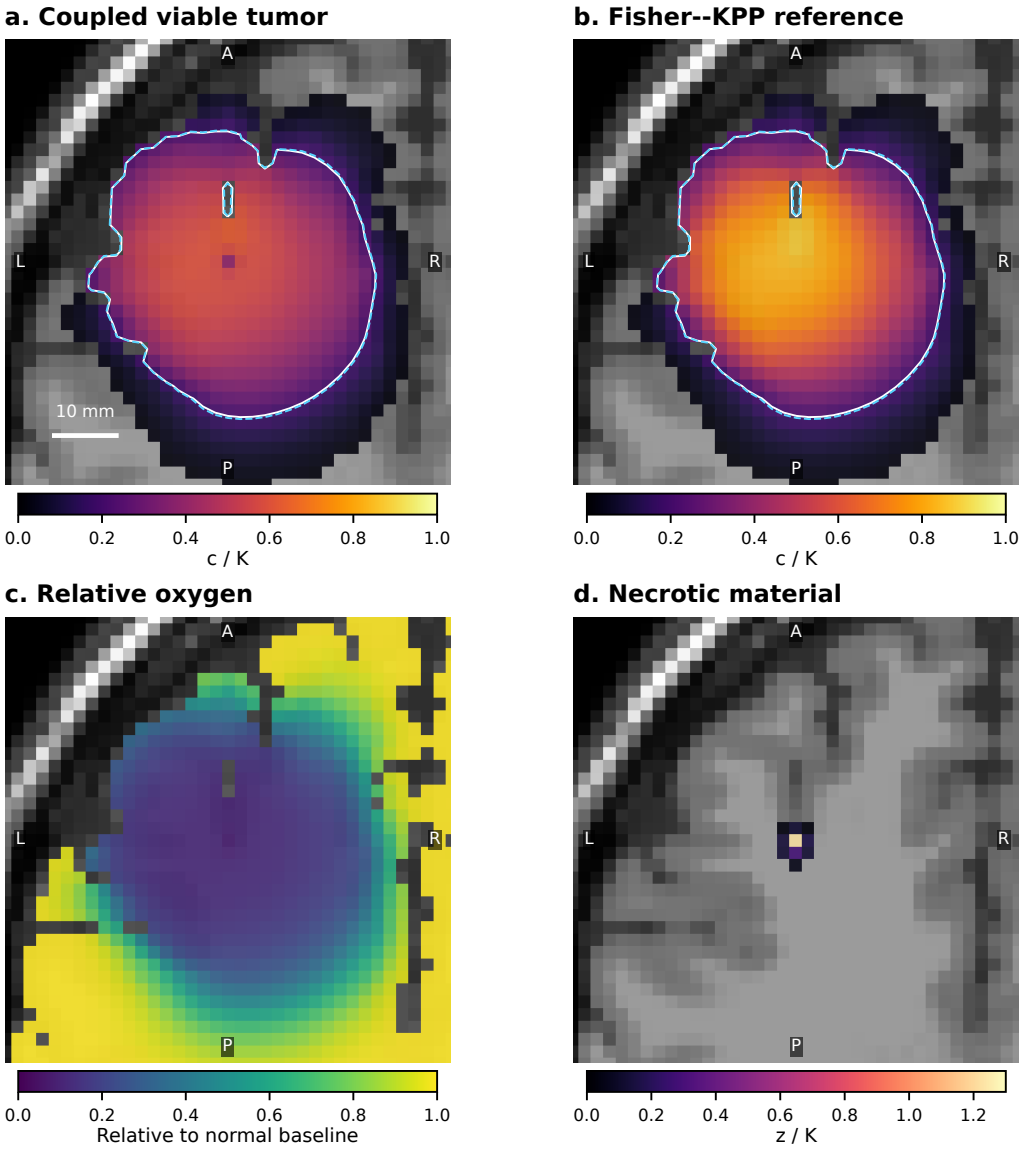


Figure 1: Day-90 fields in the matched primary simulations. Axial views at RAS $z = 30$ mm show a 72×72 mm region, with the original 2 mm simulation pixels retained. Viable density and the reference share the same 0–1 color scale; oxygen is relative to the normal baseline. Necrotic material is shown over 0–1.3 without clipping its peak at one. White solid and cyan dashed contours mark $c/K = 0.2$ in M2 and M0, respectively. Tumor/necrosis overlays below 0.02 of K are transparent for display only. The scale bar is 10 mm. These thresholds and colors have no calibrated correspondence to MRI contrast.

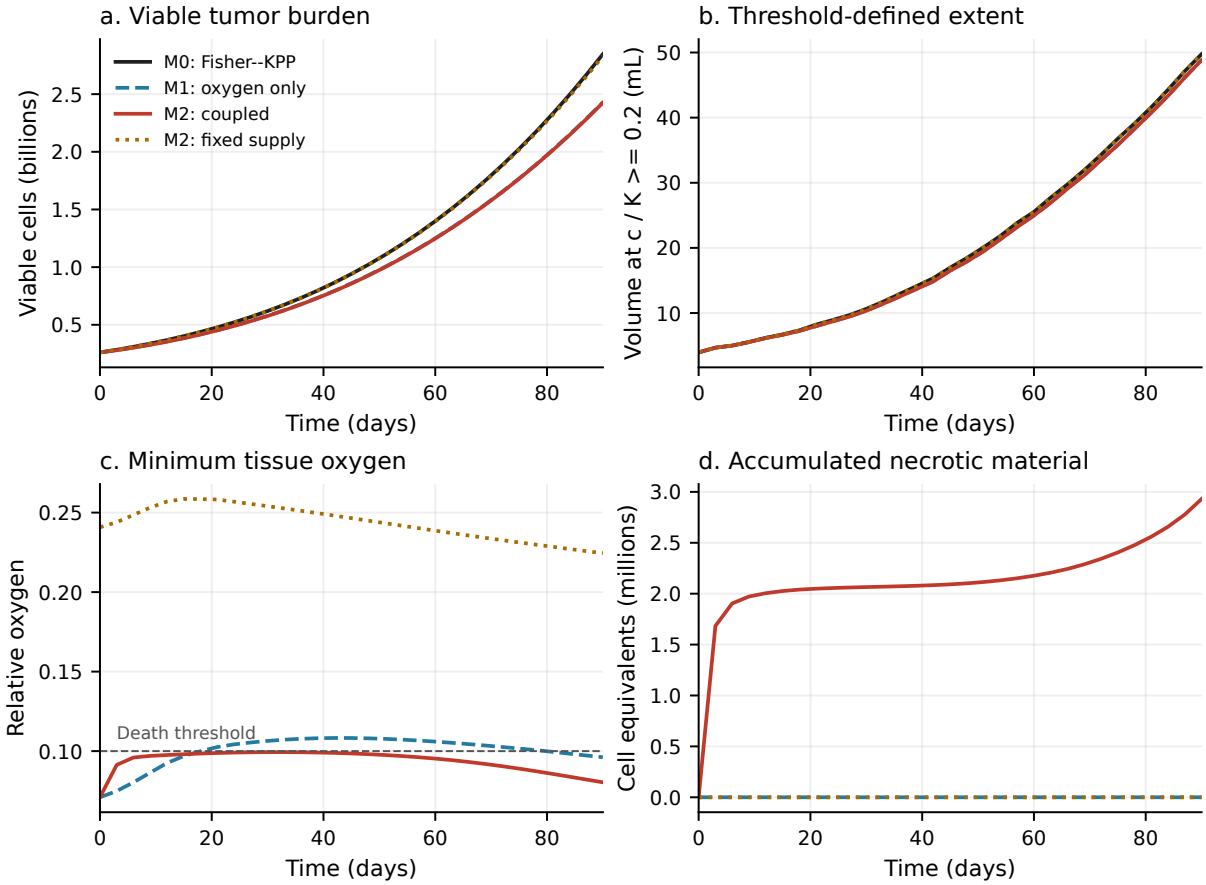


Figure 2: Time courses for the matched reference and selected ablations. Panels show viable cell count, threshold-defined viable volume, minimum tissue oxygen and necrotic cell equivalents. M0 has no oxygen or necrosis state; these curves are consequently omitted. The dashed horizontal line in panel c marks $\theta_d = 0.1$. M1 and M2 burden curves nearly overlap. All curves are deterministic simulations with assigned parameters; they are not measurements, replicate averages or uncertainty intervals.

Table 3: Day-90 outcomes under temporal and anatomical-grid refinement for M2. h is voxel spacing in mm and Δt is the maximum step in days. The 1 mm run changes both the numerical mesh and the discretized tissue boundaries.

h	Δt	$N_c/10^9$	$N_z/10^6$	$\min o$	$\max(z/K)$
2	0.25	2.4258	2.9302	0.0804	1.226
2	0.125	2.4239	2.9431	0.0798	1.266
2	0.0625	2.4230	2.9493	0.0795	1.285
1	0.125	2.4524	3.6067	0.0513	3.416

assumptions.

At 2 mm, reducing the maximum time step from 0.25 to 0.125 day decreased the final relative L^2 difference in viable density against the 0.0625-day calculation from 0.1270% to 0.0423%. Corresponding necrotic-field differences decreased from 4.238% to 1.386%. The relative oxygen differences were 0.0129% and 0.0043%. These comparisons support temporal refinement of the implemented scheme; a finite reference calculation does not provide an exact error or establish an asymptotic convergence order.

The native 1 mm calculation contained 2.452 billion viable cells, compared with 2.878 billion in its matched M0 reference, a 14.78% reduction. This relative reduction is similar to the primary result. However, minimum oxygen fell to 0.0513, the necrotic-equivalent count increased to 3.607 million and peak z/K rose to 3.416. The necrotic threshold occupied 0.032 mL (32 native voxels), while low-oxygen volume was 0.027 mL. These local quantities are sensitive to spatial resolution and to the discrete anatomy. Neither grid resolves the assigned 0.2 mm oxygen length; spatial convergence of the oxygen–necrosis subsystem has not been demonstrated.

3.4 Status of parameter estimates

No biological parameters were estimated from measurements. The reported values are forward-simulation inputs, and no posterior distributions, patient-specific confidence intervals or predictive clinical errors can be inferred from them. The ablations identify influential assumptions for this example; influence alone does not show that a parameter is recoverable from available measurements. The next section specifies the measurements, priors and withheld-data tests needed to turn the simulator into an inference workflow.

4 Discussion

4.1 Interpretation and scope of the extension

The principal result is a separation between simulated burden and threshold-defined extent. With identical assigned migration and growth parameters, oxygen feedback lowered viable density in the interior while leaving the lower-density boundary comparatively similar. Well-oxygenated regions retain Fisher–KPP-like growth, whereas the interior experiences inhibited births and, at sufficiently low oxygen, death transfer. The ablations show that burden-dependent supply suppression dominates the overall effect for this parameter set. The numerical comparison establishes how the selected equations behave; it does not establish that they predict glioma better than Fisher–KPP. Predictive model selection requires fitting each candidate to the same calibration observations and comparing their withheld-data predictions.

The extension is deliberately small, but its state interpretation matters. Immobile z with zero clearance accumulates historical cell loss, while viable cells can diffuse into the same locations. The equations therefore allow $c + z > K$ and $z > K$ locally. A normalized necrotic volume

fraction should not be inferred from z/K. The observed grid sensitivity reinforces this limitation. If physical packing, tissue displacement or a necrotic cavity becomes a target, a volume-fraction, mechanical or poromechanical formulation would be a different model requiring additional parameters and observations. Recent MRI-informed poromechanical forecasting and sequential biomechanical model-selection studies provide relevant alternatives, but the cited 2026 records are preprints and concern distinct preclinical datasets [11, 12]. They do not validate the present closure.

4.2 What measurements could identify parameters?

A practical first stage would estimate a limited growth–migration subset from serial, registered MRI, while constraining initialization and the white-to-gray migration ratio. In a homogeneous, isotropic Fisher–KPP medium, the asymptotic minimal front speed is $v = 2\sqrt{D\rho}$ and the front length scale is proportional to $L = \sqrt{D/\rho}$. These relations explain why growth velocity and spatial spread offer complementary information. They are not identities for a finite, heterogeneous brain with an unknown initial state. One snapshot with unknown tumor age generally cannot separate all dimensional rates. Recent single-scan inference methods introduce model and scaling assumptions [8]; a 2026 simulated-biopsy preprint similarly distinguishes information about infiltration length from information about growth speed [13].

MRI-derived cell-density maps can supply more information than binary segmentations only when their calibration is defensible. ADC measures water mobility, not the effective migration coefficient D. Its association with cellularity is influenced by extracellular space, edema, tissue composition and necrosis [16]. Nor should a FLAIR boundary automatically be treated as a fixed tumor-density contour: an MRI–histology study challenged that assumption in a single autopsy case [17]. In a 2025 study of image-guided biopsies, associations between MRI-derived cell-density maps and histology differed between untreated and recurrent tumors [9]. The treatment context and observation mapping must therefore be part of the inference model. Biopsy coordinates, brain shift, fixation deformation, partial volume and the conversion from two-dimensional counts to cells/mm^3 require explicit treatment.

Oxygen introduces further observation ambiguity. The state o is availability relative to a normal baseline; it is not directly oxygen extraction fraction, hemoglobin saturation or oxygen tension. Oxygen-enhanced MRI, BOLD-based measures and hypoxia PET could constrain an observation model, but their signals depend on delivery, vascular and tissue factors. A histological comparison of MRI-derived oxygen measures illustrates that an imaging surrogate cannot simply be equated with a hypoxia marker [18]. The 2026 glioma habitat repeatability study is encouraging for measurement consistency, but repeatability does not establish an absolute oxygen calibration or biological specificity [10].

Priors are necessary where these measurements leave parameters weakly identified. Positive rates and lengths can use log-normal priors; ordered oxygen thresholds can use a transformed prior satisfying $0 < \theta_d < \theta_p < 1$. Hyperparameters should come from independently evaluated experiments or a defensible population model, with uncertainty reflecting transfer between systems. The working values in Table 1 are not empirical prior means. Prior predictive checks should reveal implausible growth, oxygen depletion and accumulation above the intended interpretation of K. Sensitivity analysis, profile likelihoods and posterior correlations can then identify parameter combinations that the observations fail to distinguish. The 2025 identifiability/model-selection study provides a relevant framework, although its virtual-cohort chemoradiation models differ from this system [6]. A narrow posterior produced mainly by a strong prior should not be described as precise measurement-based estimation.

Table 4: Proposed measurement and prior strategy. These are candidate information sources, not measurements acquired or estimates obtained in this study. Each modality requires its own observation operator and uncertainty model.

Quantity	Potential information	Restriction or prior needed
$c(0)$, D, ρ	Serial T1/T2/FLAIR, diffusion-weighted and quantitative MRI; registered anatomy	Joint initialization uncertainty; estimate a small parameter subset; constrain tissue ratios. MRI water diffusion is not cell migration.
K and cell scale	Spatially localized tumor-cell counts with tissue-volume or stereological calibration	Distinguish tumor from non-tumor cells and correct sampling/deformation; MRI alone does not fix the absolute scale.
β, α	Oxygen-sensitive imaging combined with perfusion/vascular measurements and externally calibrated uptake information	Supply and consumption are confounded; blood-flow surrogates are not the restoration coefficient itself.
ℓ_o	External transport measurements or multiscale vascular/oxygen models	Constrain externally when subvoxel; the current 1–2 mm meshes do not resolve 0.2 mm.
θ_p, θ_d, δ	Controlled oxygen-dependent growth and survival experiments; spatially matched hypoxia/viability observations	Relative-oxygen scale must be calibrated. Ki-67 is a cycling fraction, not directly a growth rate; hypoxia markers are not absolute oxygen readings.
λ_z and z mapping	Longitudinal necrotic-region imaging plus localized pathology	Separate formation from clearance and cell equivalents from tissue volume; a single scan cannot determine both histories.

4.3 Uncertainty quantification and informative measurements

Parameter estimation should produce joint uncertainty distributions rather than a single best fit. A Bayesian analysis could combine independently justified priors with an observation likelihood accounting for imaging noise and spatial correlation. Hierarchical population priors could support patients with sparse imaging, provided the population model is learned within training data. Uncertain initialization, registration and segmentation should be included as nuisance quantities or evaluated through repeated analyses. Strong posterior correlations and sensitivity to alternative priors would indicate which rates or combinations remain poorly constrained.

Forecast ensembles should propagate these uncertainties into viable-burden intervals, infiltration maps and oxygen/necrosis predictions. Measurement uncertainty, numerical error and model inadequacy require separate assessment: a narrow conditional posterior does not account automatically for a wrong oxygen closure or unresolved spatial gradients. Competing Fisher–KPP and coupled formulations should be evaluated separately on withheld observations; any model-averaging weights should be learned without those evaluation data. Posterior predictive checks, interval coverage and width, and sensitivity to the observation operator should accompany point-error metrics.

Pash et al. developed a scalable Bayesian reaction–diffusion framework using longitudinal imaging and investigated imaging frequency and experimental design [19]. This provides a methodological reference for the proposed inference workflow, not validation of the present oxygen–necrosis extension. Once uncertainty is calibrated, candidate scan times or additional modalities could be ranked by their expected reduction in forecast uncertainty or their expected value for distinguishing treatment options. Additional measurements are useful when they resolve a consequential ambiguity, rather than merely increasing the number of fitted parameters.

4.4 A validation pathway with independent targets

Validation must be tied to a specified observable, population and prediction horizon. Five complementary stages are appropriate here.

1. **Verify the solver.** Check conservation, positivity, analytical limits, solver tolerances and time/space refinement. These checks address mathematical implementation. The present unresolved oxygen length and necrotic peak require further spatial assessment before local quantitative claims.
2. **Test synthetic recovery.** Generate noisy observations with known parameters, realistic imaging resolution and registration errors, then assess recovery and uncertainty coverage. Use a different mesh, solver or observation formulation to reduce the inverse crime of fitting data generated by the identical numerical pipeline. Synthetic success does not establish biological adequacy; earlier MRI-constrained reaction–diffusion work demonstrated the importance of separating synthetic recovery from in vivo forecasting [20].
3. **Forecast withheld observations.** Fit early observations, freeze the calibration and measurement mapping, and predict later scans over a declared horizon. Compare separately calibrated M0, M1 and M2 with a simple growth baseline. Treated tumors require explicit treatment terms or a justified untreated interval. Updating with each new scan is sequential assimilation, a different evaluation from a frozen forecast. The 2025 assimilation study illustrates the distinction in high-grade glioma during chemoradiation [7].
4. **Check latent fields independently.** Reserve spatially localized cell counts, viable/necrotic histology and oxygen-related observations for evaluation rather than using all samples for calibration. Report localization and measurement uncertainty. A successful volume forecast alone does not validate o, z or the death mechanism. Controlled preclinical oxygen or perfusion perturbations would provide stronger tests of the proposed feedback than morphology alone.

5. **Evaluate transportability.** Apply a fixed analysis and prior strategy to an external cohort, with any permitted patient-specific calibration specified in advance. Group splits by patient and, where relevant, site and treatment status. Prospective clinical utility is a subsequent question, beyond retrospective image agreement.

Evaluation should include volume errors, surface distances and overlap, together with quantitative field errors where calibrated measurements exist. Report predictive interval coverage and width, not only point accuracy. Repeated biopsies within a patient and spatially correlated voxels are not independent replicates; patient-level or hierarchical analyses should reflect this dependence. The 2025 biopsy-validation study is a useful example of independent tissue evaluation and context-dependent agreement [9]. An accurate forecast can coexist with non-unique physiological parameters, while an identifiable model can still have the wrong biology. Both prediction and parameter interpretation must be assessed.

Clinical validation should subsequently use a prespecified population, prediction horizon and analysis, with external testing before prospective, blinded forecasting. Imaging endpoints should account for treatment effects and pseudoprogression using a defined response-assessment framework such as RANO 2.0 [21]. Agreement with a later MRI or recurrence location is evidence about prediction under the observed treatment; it does not establish the outcome under an alternative treatment. Demonstrating clinical utility would require prospective comparative evaluation of decisions and, ultimately, controlled trials assessing survival, neurological function, quality of life and toxicity.

4.5 Treatment evaluation and candidate strategies

The present simulations suggest why treatment evaluation should consider viable burden and spatial infiltration together: similar threshold-defined volumes can conceal different modeled interiors. Their difference is not a treatment effect, because no therapy was simulated. Extending the framework would require an explicit resection geometry and residual disease state, spatially delivered radiation and drug exposure/response terms. Surgery may also change anatomy and perfusion, requiring updated transport and observation models. Natural growth and treatment-response parameters should be estimated jointly from appropriately timed observations, or constrained independently when they cannot be separated.

An oxygen-dependent radiation-survival model could test the consequences of hypoxia and reoxygenation between fractions, but the relative availability o first needs a calibrated connection to the relevant radiobiology. Neither c nor z currently represents a measured clonogenic population or radiation-induced normal-tissue injury. Computing a tumor-control probability would require additional assumptions about clonogenic fraction and survival; evaluating toxicity would require a separate normal-tissue model. Treatment-related necrosis and imaging changes must be distinguished from spontaneous hypoxic loss. These additions are proposed mechanisms requiring independent experiments, not interpretations already supported by the three simulated fields.

Recent studies illustrate complementary evaluation targets. Balcerak et al. evaluated imaging- and physics-informed infiltration estimates against subsequent recurrence in a dataset of 152 glioblastoma patients [22]. Häger et al. examined model-informed target delineation and voxel-based dose planning in three cases, demonstrating planning feasibility while identifying difficulties in reproducing complex dose distributions [23]. These retrospective prediction and planning studies do not establish a survival benefit from delivering the proposed alternatives. Earlier risk-aware radiotherapy optimization used a cohort of 100 virtual patients [24], illustrating how uncertainty can inform simulated treatment comparisons without constituting a clinical trial.

A next experimental stage could use paired virtual cohorts to compare a specified standard-treatment comparator with adaptive fractionation, spatial dose painting, hypoxia-modifying combinations and alternative drug/radiation sequences. Each candidate would require an explicit

mechanism and independently constrained response parameters. Comparisons should use matched dose/resource constraints, assess normal-brain exposure and delivery feasibility, and test whether a favorable ranking persists across parameter and model uncertainty. Adaptive policies must use only observations available at the decision time. Outcomes could include viable burden, spatial recurrence and time to a prespecified progression criterion; a survival prediction would require a separately validated link to that endpoint. Controlled preclinical oxygen/perfusion perturbations could test the proposed feedback before patient translation. Virtual cohorts should prioritize experimentally testable hypotheses and informative measurements; they cannot identify an effective new therapy solely from a favorable simulated result.

4.6 Limitations and staged roadmap

The study uses one healthy anatomical phantom, one seed and one primary parameter set. The ablations and refinement runs are not a population analysis. Scalar migration omits fiber orientation, and no displacement, edema, vascular evolution, immune interactions, phenotype switching or therapy is modeled. Quasi-steady oxygen identifies effective coefficient groups rather than absolute oxygen equilibration rates. Minimum oxygen and necrotic accumulation are under-resolved, and zero clearance accentuates accumulation. Finer simulation grids do not create additional measurement information.

The following sequence makes the research roadmap concrete; all stages beyond the reported numerical comparisons remain future work.

1. **Establish an estimable MRI model.** Resolve the numerical limitations, define registered imaging/field observation operators and demonstrate synthetic recovery of a small migration–proliferation subset. Add oxygen/death parameters only when independent measurements or justified priors constrain them.
2. **Deliver calibrated uncertainty.** Infer joint parameter distributions, propagate initialization and measurement uncertainty, and report forecast intervals with coverage checks, prior sensitivity and a numerical/model-error assessment.
3. **Demonstrate external forecasting.** Fit early scans, freeze the analysis and predict later observations; compare separately calibrated model variants and simple baselines, then repeat on an external cohort with independent spatial biological targets.
4. **Evaluate treatment hypotheses.** Implement and verify treatment and normal-tissue effects, calibrate their response parameters, and compare feasible strategies in virtual cohorts and controlled experiments, including informative imaging schedules.
5. **Test clinical utility.** Prespecify prospective, blinded forecasts before evaluating model-guided decisions in comparative studies and controlled trials with patient-centered benefit and harm endpoints.

Add complexity only to address predictive discrepancies supported by independent observations. Each stage requires its own evidence.

4.7 Conclusion

A minimal oxygen–necrosis extension can be implemented alongside Fisher–KPP within the same anatomical and interactive workflow. Under the assigned conditions, the extension reduced viable burden much more than low-threshold extent, largely through growth inhibition. Its localized necrotic predictions were sensitive to grid spacing and cannot be interpreted as validated tissue fractions. The resulting code, recorded experiments and manuscript provide a reproducible starting point for measurement-based inference and staged validation, rather than a patient-calibrated predictive model.

Code, data and supplementary material

The manuscript, source code, interactive Jupyter notebooks, recorded simulations and synchronized visualizations are available at https://github.com/arvidl/brainweb_fisher_kpp. The manuscript project includes LaTeX/BibTeX sources, simulation and figure-generation scripts, machine-readable experiment records, source and data hashes, and numerical-test results. Repository history and the recorded source hashes identify the implementation used for this study. An immutable release identifier and archival DOI should accompany a submitted version when available. BrainWeb attribution and source records are retained separately from the software.

Study scope and declarations

All experiments are computer simulations on an anatomical phantom. No patient measurements, new human sampling or new animal experiments were used. Biological parameter inference and external validation remain proposed work. Authorship, affiliations, funding, competing interests, contributions and any required AI-use declaration must be finalized before journal submission.

A Anatomical data and initialization

The files are `data/t1_icbm_normal_1mm_pn0_rf0.nii.gz` and `data/phantom_1.0mm_normal_crisp.nii.gz`. The former is the BrainWeb T1 alias with 1 mm spacing, zero added noise and zero intensity nonuniformity; the latter is the matching original normal crisp phantom, converted from MINC to NIfTI. These are not the separate BrainWeb 20-subject anatomical models. Both native arrays have shape $181 \times 217 \times 181$ and the same RAS+ affine

$$A = \begin{pmatrix} 1 & 0 & 0 & -90 \\ 0 & 1 & 0 & -126 \\ 0 & 0 & 1 & -72 \\ 0 & 0 & 0 & 1 \end{pmatrix}. \tag{8}$$

Native gray- and white-matter voxel counts are 902,912 and 674,777. Tissue labels 2, 3 and 8 define the active domain; label 8 uses gray-matter coefficients. CSF is label 1 and is excluded. No tumor is present in the source images. T1 intensities are divided by their 99.8th percentile and clipped for background display only; they do not determine transport or growth coefficients. At 2 mm the affine diagonal is $(2, 2, 2, 1)$ with the same translation, and array shape is $91 \times 109 \times 91$.

The SHA-256 checksums of the bundled compressed files are:

T1: `aa40262b2702653188fd51bdec844a393b825650fe1155a84d8606f5dfd99eb3`
Labels: `afdff78fc1b06ad7c7d8216cf96be737a1450ef5b6abf27eb732c0bf8b66fac0`.

The manifest retains official aliases and source URLs. Regenerating a MINC-to-NIfTI conversion can change compression/header encoding and therefore file hashes; a replacement requires an explicit voxel/affine comparison and updated manifest. BrainWeb attribution and terms remain applicable to these data.

For $\boldsymbol{x}_0 = (-30, 25, 30)$ mm and $\sigma_k = 6$ mm, let $r^2 = \sum_{k=1}^{3}[(x_k - x_{0k})/\sigma_k]^2$. The initialization is

$$c(\boldsymbol{x}, 0) = 80{,}000 \exp(-r^2/2)\, \mathbf{1}_{\{r^2 \le 9\}}\, \mathbf{1}_{\Omega}(\boldsymbol{x}), \qquad z(\boldsymbol{x}, 0) = 0. \tag{9}$$

Oxygen is initialized by the elliptic solve. The physical center is retained when grids change. The nearest saved slice indices on the 2 mm grid are $(30, 76, 51)$, corresponding to RAS $(-30, 26, 30)$ mm: the saved coronal plane is at 26 mm, rather than the off-grid 25 mm seed coordinate. Figure 3 supplies orthogonal views of the same fields as Figure 1.

B Numerical implementation and verification

B.1 Conservative transport and reaction splitting

For neighboring active cells i, j on a Cartesian grid of spacing h, use the harmonic face coefficient

$$D_{ij} = \begin{cases} 2D_iD_j/(D_i + D_j), & D_i + D_j > 0, \\ 0, & \text{otherwise}, \end{cases} \qquad w_{ij} = D_{ij}/h^2. \tag{10}$$

Faces leaving the active domain are assigned zero weight, as are external volume faces. A cell-transport step is

$$c_i^* = c_i^n + \Delta t \sum_{j \in \mathcal{N}(i)} w_{ij}(c_j^n - c_i^n), \qquad \Delta t \le \frac{0.9}{\max_i \sum_j w_{ij}}. \tag{11}$$

Symmetric fluxes conserve total cell count under pure diffusion. The step bound is sufficient for nonnegative explicit updates. Faces are stored without periodic wrapping; no density clipping

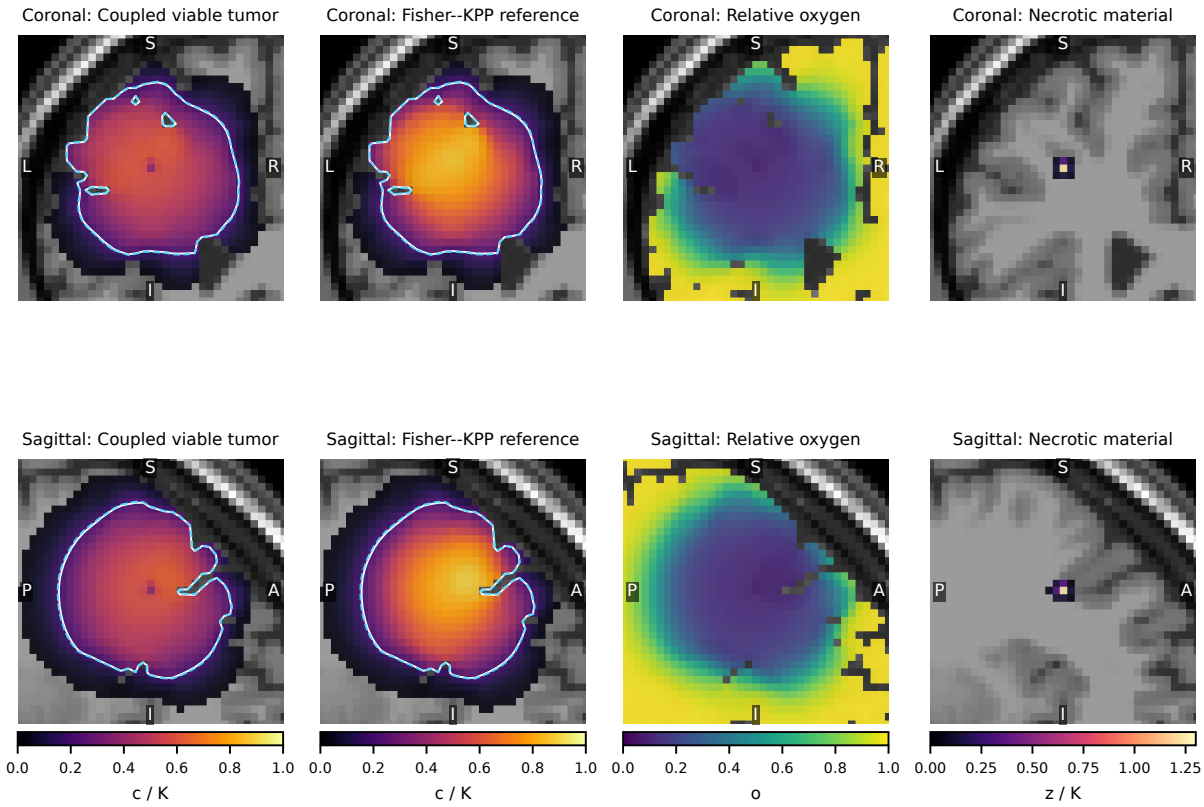


Figure 3: Orthogonal day-90 comparisons through the saved seed planes: coronal $y = 26$ mm (top) and sagittal $x = -30$ mm (bottom). Each panel spans 72×72 mm with native 2 mm pixels. Columns show coupled viable density, the matched Fisher–KPP reference, relative oxygen and necrotic material. The viable and reference scales are identical, and the necrotic scale extends to 1.3. White solid and cyan dashed contours indicate the respective $c/K = 0.2$ boundaries.

is applied. Requested steps are subdivided as needed, and steps end at the exact saved output times.

For M0, the reaction following diffusion is the exact logistic flow

$$c_i^{n+1} = \frac{c_i^*}{\exp(-\rho_i \Delta t) + (c_i^*/K_i)[1 - \exp(-\rho_i \Delta t)]}. \tag{12}$$

For M2, freeze oxygen at o^n and necrosis at z^n during birth. Set $F_i = K_i - z_i^n$. If $F_i > 0$ and $c_i^* < F_i$, use

$$r_i = \rho_i g(o_i^n) F_i / K_i, \qquad c_i^b = \frac{c_i^*}{\exp(-r_i \Delta t) + (c_i^*/F_i)[1 - \exp(-r_i \Delta t)]}. \tag{13}$$

Otherwise $c_i^b = c_i^*$. With $q_i = \delta h(o_i^n)$, the subsequent death/clearance step is

$$L_i = c_i^b[1 - \exp(-q_i \Delta t)], \quad c_i^{n+1} = c_i^b - L_i, \quad z_i^{n+1} = (z_i^n + L_i)\exp(-\lambda_z \Delta t). \tag{14}$$

The same L_i leaves c and enters z, preserving cell equivalents when clearance is zero. Exact frozen-coefficient substeps do not make the full coupled update exact: diffusion, birth, death and clearance are Lie-split, and oxygen is lagged. The complete method is first order in time. The centered spatial stencil has second-order consistency in smooth regions, without a global spatial-order claim at sharp tissue boundaries. Tumor and necrosis arrays use float32 and compiled Numba CPU kernels.

B.2 Quasi-steady oxygen solve

Let L_h be the unit-diffusivity no-flux Laplacian on the active voxel graph, $u = c/K$, and $p_i = [1 + \alpha(c_i + z_i)/K_i]^{-1}$. After initialization and each tumor update, solve

$$\left[-\ell_o^2 L_h + \mathrm{diag}(p + \beta u)\right] \boldsymbol{o} = \boldsymbol{p}. \tag{15}$$

The positive reaction diagonal makes this system symmetric positive definite, including disconnected tissue components. Matrix-free conjugate gradients use float64 arithmetic, the previous oxygen state as the initial guess and a Jacobi preconditioner. The relative residual tolerance is 10^{-7}, with a 200-iteration maximum per solve. Nonconvergence or oxygen-bound failure raises an error. The zero-uptake and zero-diffusion limits have direct solutions. Stored oxygen is float32. Outside-domain zeros are excluded-domain sentinels and are excluded from oxygen statistics and overlays.

The primary M2 run required 361 solves and 873 total conjugate-gradient iterations; the largest recorded relative residual was below 10^{-7}. This residual measures algebraic solution accuracy, not accuracy of the continuum approximation on the chosen grid.

B.3 Recorded checks and refinement comparisons

The 35-test suite includes exact logistic growth; reflecting-boundary diffusion modes; heterogeneous-flux conservation; impermeable interfaces and no wrapping; positivity and unsafe-step rejection; parameter checks; exact output times; data affine/tissue checks; uniform and nonuniform oxygen solutions; oxygen barriers; exact death transfer and clearance; occupied-capacity behavior; Fisher–KPP recovery; live-plus-dead balance; coupled temporal refinement against an independent Runge–Kutta calculation; failed-solver reporting; rendering; and replay compatibility. Test results are stored in `paper/data/pytest.xml`. Notebook smoke checks and media review are separate interface checks.

For field v, final relative L^2 differences are $\|v_{\Delta t} - v_{0.0625}\|_2 / \|v_{0.0625}\|_2$ on the same 2 mm grid (Table 5). The denominator is nonzero in these cases. These are finite-reference differences, not rigorous continuum error bounds. The 1 mm assessment is reported in Table 3; because the tissue labels also change under resampling, it is not used to infer spatial order.

Table 5: Final relative L^2 differences (percent) against the 0.0625-day M2 calculation on the 2 mm grid.

Δt (days)	Viable tumor	Oxygen	Necrotic material
0.25	0.1270	0.0129	4.238
0.125	0.0423	0.0043	1.386

C Inverse formulation, priors and identifiability

C.1 An example estimation objective

With the observation model in Equation (7), a possible maximum-a-posteriori objective for continuous approximately Gaussian observations is

$$(\widehat{\boldsymbol{\theta}}, \widehat{\boldsymbol{\eta}}) = \arg\min_{\boldsymbol{\theta},\boldsymbol{\eta}} \ \mathcal{J}(\boldsymbol{\theta}, \boldsymbol{\eta}), \tag{16}$$

$$\mathcal{J} = \frac{1}{2}\sum_{m,j} \boldsymbol{r}_{mj}^{\mathsf{T}} \Sigma_{mj}^{-1} \boldsymbol{r}_{mj} - \log \pi(\boldsymbol{\theta}, \boldsymbol{\eta}), \qquad \boldsymbol{r}_{mj} = \mathcal{H}_m[\boldsymbol{s}_{\boldsymbol{\theta}}(t_j); \boldsymbol{\eta}_m] - \boldsymbol{y}_{mj}, \tag{17}$$

where $\boldsymbol{s} = (c, o, z)$ and Σ models measurement covariance. Cross-time or cross-modality correlations require a joint likelihood rather than independent terms. Segmentations may need a probabilistic classification likelihood rather than Gaussian residuals. Registration, partial volume, scanner bias, initial fields and uncertain state-to-signal mappings should enter the likelihood or be propagated as nuisance uncertainty. Bayesian sampling can quantify uncertainty; an optimizer alone does not provide calibrated predictive intervals. No such fit was performed in this study.

C.2 Important structural and practical ambiguities

1. **Absolute cell scale.** In terms of $u = c/K$ and $w = z/K$ with uniform K, the normalized dynamics do not determine K. Scaling (c, z, K) together leaves (u, w, o) unchanged. An observation dependent only on normalized density cannot identify the absolute cell count without external calibration.
2. **Age and rates.** For an unknown elapsed tumor age, multiplying D, ρ, δ and λ_z by a common factor and dividing the elapsed time by that factor leaves the autonomous state trajectory at the observation unchanged. Known longitudinal time intervals can break this particular scaling, although seed and parameter confounding may remain.
3. **Oxygen-growth coupling.** Where all observed cells remain below θ_p, the birth term depends on ρ/θ_p rather than on those parameters separately, for a fixed oxygen field. Oxygen threshold calibration and observations spanning informative regimes are needed.
4. **Delivery and uptake.** Neglecting spatial oxygen transport locally gives

$$o = \frac{1}{1 + \beta u[1 + \alpha(u + w)]}. \tag{18}$$

   One local state constrains an effective supply–uptake combination, not separate mechanisms. Spatially varying, calibrated oxygen observations across burdens could help, but do not guarantee recovery. An exchange length far below measurement and mesh resolution can be practically unidentifiable.
5. **Quasi-steady reduction.** If a dimensional oxygen equation is divided by a baseline restoration rate, ℓ_o^2 and β represent coefficient ratios. Steady observations alone cannot identify an independent absolute oxygen relaxation time removed by that reduction.
6. **Necrotic history.** For known $q = \delta h(o)$,

$$z(t) = z(0)e^{-\lambda_z t} + \int_0^t q(s)c(s)e^{-\lambda_z(t-s)}\,\mathrm{d}s. \tag{19}$$

A single terminal necrotic measurement cannot generally separate formation history and clearance. Zero clearance is a model choice here, not an estimated absence of biological removal.

Positive parameters can be transformed as $\log \theta_k \sim \mathcal{N}(\mu_k, \sigma_k^2)$, with dimensional reference units stated. For thresholds, use $\theta_p = \text{logit}^{-1}(a)$ and $\theta_d = \theta_p \,\text{logit}^{-1}(b)$. A zero-clearance component or other exact-zero ablation requires an explicit point mass or a separate model, rather than a continuous positive prior. Neither prior hyperparameters nor population distributions are supplied by the current phantom experiment.

Before a full fit, evaluate scaled observation sensitivities, their singular values, profile likelihoods and posterior dependence across plausible initial states and observation mappings. The number of voxels is not the number of independent constraints. Begin with a global migration scale and growth coefficient, restrict other quantities using external information, and add freedom only when independent observations support it. Identifiability and predictive comparison should be repeated for each candidate model and intended measurement set.

D Reproducibility and interactive artifacts

`paper/run_experiments.py` runs the nine model/step/grid cases using `examples/coupled.json`, verifies the bundled data and checks that the primary viable-count trajectory matches the saved example. It records final metrics and settings in `experiments.json`, time courses in `timecourses.csv`, lossless final seed-plane fields in `day90_planes.npz`, and provenance in `provenance.json`, all under `paper/data/`. Source hashes include the model, data loader, renderer, configuration, lockfile and experiment script. The experiments were generated before their implementation changes were committed, so provenance records working-file hashes alongside the base Git revision. The hashes identify the simulation sources independently of that earlier revision.

`paper/make_figures.py` reads these records to generate figures, table rows and numerical macros used directly by the manuscript. The local Python environment is defined by `pyproject.toml` and `uv.lock`. A `Makefile` rebuilds the figures and PDF, with a separate target to rerun the experiments. The native and 2 mm examples are deterministic; they do not sample biological uncertainty.

`tumor_growth_explorer.ipynb` exposes both models; `coupled_example.ipynb` presets the 90-day comparison. Time and RAS slices are shared across the four-field view. Live histories permit arbitrary 3D slicing, while compact replay files retain the three seed-plane histories and final full volumes. The slower comparison MP4 uses 400 frames at 20 frames/s, including a 120-frame day-90 hold; playback duration is distinct from simulated time. Synchronized movies help inspect output but are not quantitative validation. Manuscript figures retain the full computed necrotic range, unlike a display limited to 0–1.

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